SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Tucatinib + capecitabine + trastuzumab

Advanced / metastatic · previously treated HER2-positive breast cancer · NCCP 00936 · version 1 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
The three medicines work together to control previously treated HER2-positive advanced breast cancer. Treatment is reviewed regularly for benefit, symptoms and side effects.

Schedule
Tucatinib twice daily every day; capecitabine twice daily on Days 1–14 then off on Days 15–21; trastuzumab every 21 days by IV infusion or a locally approved under-the-skin injection. Continue while beneficial and tolerated, with clinician-approved changes.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Required blood tests and cardiac monitoring discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Tucatinib
HER2 TYROSINE KINASE INHIBITOR

- ☐ Diarrhoea and dehydration
- ☐ Liver test changes or liver injury
- ☐ Nausea and fatigue
- ☐ Clinically important medicine interactions

Capecitabine
ORAL CHEMOTHERAPY

- ☐ Diarrhoea / dehydration
- ☐ Hand-foot syndrome
- ☐ Mouth ulcers / reduced intake
- ☐ Low blood counts and infection
- ☐ Cardiotoxicity including chest pain / ischaemia
- ☐ Severe early toxicity including DPD-related toxicity
- ☐ Renal impairment and medicine interactions

Trastuzumab
HER2-TARGETED ANTIBODY

- ☐ Heart pumping-function reduction / heart failure
- ☐ Infusion or allergic reaction
- ☐ Diarrhoea and fatigue
- ☐ Rare serious lung effects

Discuss the active phase, supportive medicines, fertility / menopause, treatment alternatives, monitoring, urgent contacts and when the plan will be reviewed.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00936 v1.

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